Accelerating Clinical Data Exchange™

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Submitted electronically via *https://www.regulations.gov*

The Honorable Dr. Mehmet Oz
Administrator, Centers for Medicare & Medicaid Services
200 Independence Avenue, SW
Washington, D.C. 20201

Dr. Thomas Keane
Assistant Secretary for Technology Policy and National Coordinator for Health Information Technology
330 C St. SW, Floor 7
Washington, D.C. 20201

RE: Request for Information; Health Technology Ecosystem [CMS-0042-NC; RIN 0938-AV68]

Dear Administrator Oz and Assistant Secretary Keane:

MRO Corporation (MRO) appreciates the opportunity to provide comments in response to the published Request for Information (RFI) regarding the Health Technology Ecosystem (CMS-0042-NC).[1] MRO is a leading healthcare technology company that facilities the exchange of clinical data between hospitals, clinics, payers, and other users of clinical data. Our solutions include, but are not limited to, quality reporting, payment and audit validation, Healthcare Effectiveness Data and Information Set (HEDIS) reporting, and Release of Information (ROI). MRO partners with hospitals, health systems, and physician practices across the country on their ROI solution for the secure, compliant, and efficient exchange of Protected Health Information (PHI). KLAS Research has named MRO the #1 provider of ROI services for eleven years, reflecting direct client feedback and reviews. We are connected to over 1,100 hospitals, 35,000 clinics, 170,000 individual practitioners, and 250+ electronic health record (EHR) and health IT systems.

MRO strongly supports the steps the Centers for Medicare & Medicaid Services (CMS or the Agency) and the Assistant Secretary for Technology Policy/National Coordinator for Health Information Technology (ASTP/ONC) are taking to modernize the nation's digital health ecosystem through greater access to innovative health technologies. As a clinical data management company, MRO performs an essential role for healthcare providers to facilitate the Health Insurance Portability and Accountability Act (HIPAA)-compliant release of health information to patients and authorized requesters, navigating the complex and expanding digital environment. MRO

[1] 90 FR 21034 (May 16, 2025).

appreciates CMS and ASTP/ONC leadership and shares the goals of the RFI to examine ways to best advance a seamless, secure, and patient-centered digital health infrastructure.

MRO is committed to protecting the privacy and security of patient PHI and providing secure, compliant, and efficient patient access to health information. We support efforts to improve care quality and patient experience through innovation and enhanced interoperability. However, increased data sharing must not compromise privacy. Patient, provider, and payer trust in the security and privacy of health information is paramount to achieving enhanced data sharing and interoperability across the healthcare ecosystem. Without strong oversight, application programming interfaces (APIs) can expose PHI beyond patient intent or awareness. At the same time, privacy and security concerns should not be used to unnecessarily block the flow of information. We believe that APIs should be enabled with safeguards that enforce HIPAA protections.

The below Executive Summary highlights the key themes of our comments, and our detailed responses (numbered in accordance with the RFI) follow.

EXECUTIVE SUMMARY

To enhance data sharing and interoperability, while ensuring that patient health information remains private and secure, we believe that policymakers should focus efforts on:

- Revisiting the existing information blocking rules with a focus on how to better enable the exchange of health data while maintaining rigorous protections for PHI, and in a way that drives fair competition and private sector innovation.

- Ensuring new technological innovations, including those using large language models, adhere to or exceed existing HIPAA Standards and honor the intent of HIPAA with particular focus on patient privacy protection concepts such as consent, minimum necessary and sensitive personal health information. This is particularly important when that information is transferred outside the realm of HIPAA Covered Entities and their Business Associates.

- Solidifying the foundational elements of clinical data exchange and management directly associated with authenticated patient access, consent, and patient care, through provider-to-provider exchange.

We believe that these initiatives, taken together, will advance CMS's and ASTP/ONC's collective goal of improving the quality of healthcare information exchange, will drive innovation in the private sector, and will ultimately improve patient care.

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E. TECHNOLOGY VENDORS, DATA PROVIDERS, AND NETWORKS

1. Ecosystem

TD-1. What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

We see significant demand for digital products to support Medicare beneficiaries and caregivers. Nonetheless, products have not realized their full potential because the data available for these products is often incomplete or inaccessible. Because of this, MRO believes it is critical that CMS maintain and enforce policies that promote information sharing and prevent information blocking while maintaining information privacy and security. If CMS enables secure data exchange, we are confident that the market will produce viable solutions for Medicare beneficiaries and caregivers. However, we do not think it would be a productive use of resources for CMS to promulgate additional government regulation mandating the use of particular APIs or other technologies.

TD-2. Regarding CMS Data, to stimulate developer interest—

> ***a. What additional data would be most valuable if made available through CMS APIs?***
>
> ***b. What data sources are most valuable alongside the data available through the Blue Button 2.0 API?***
>
> ***c. What obstacles prevent accessing these data sources today?***
>
> ***d. What other APIs should CMS and ASTP/ONC consider including in program policies to unleash innovation and support patients and providers?***

MRO appreciates CMS interest in improving the use of APIs. We believe that the way to encourage the use of APIs is through compelling use cases and requiring APIs to make available a richer data set for innovative clinical decision support tools and other valuable clinical use cases. We believe one of these compelling use cases would be reducing the burden of administrative reporting. Moving to fully digital administrative data exchange would spur providers to adopt this technology and would generate positive spillover effects into other compelling API use cases.

For example, Electronic Submission of Medical Documentation (esMD) submissions still rely on an interface with inadequate traceability of transactions and failure reporting. Providers and Health Information Handlers (HIHs) are required to build a CONNECT-compatible gateway to submit documentation through esMD, which can be technically challenging and resource-intensive. We also see a greater need for API adoption in quality submissions, in which many providers are still

required to upload pdfs manually to web portals. CMS efforts on Electronic Clinical Quality Measures (eCQMs) are a good step in moving toward digital measure submission from EHRs. Lastly, the Data Aggregator Validation (DAV) program administered by the National Committee for Quality Assurance (NCQA), whose purpose is to certify the digital transfer of data for HEDIS reporting, requires manual and analog validation processes for vendors to become certified. Accordingly, we recommend that CMS address the inadequacies of these existing operational and administrative processes to advance high-quality data exchange.

2. Digital Identity

TD-3a. Regarding digital identity implementation: What are the challenges and benefits?

MRO believes that widespread adoption of digital identity verification would improve data security and improve efficiency, as long as the framework aligns with existing HIPAA best practices. The major barrier to digital identity verification is the lack of common standards for trust in digital identity verification across different technologies. Today, even in the same hospital, different systems do not recognize the digital identity verification of other systems, requiring duplicative credentialing and confusing verification processes as patients traverse through their care journey. Dramatic efficiencies could be realized if an enforceable and interoperable standard of identity could be created, such that a digital identify verification on one system would be trusted on another.

In the market today, we see huge variations in identity verification standards, with some technologies relying on a variety of sources including phone records, death master files, credit records, and various other personal identifiers. This lack of standardization creates problematic variation in accuracy and validity of identities. This can lead to a situation in which layered identity requirements can simultaneously verify, and then fail to verify, an individual's identity.

TD-3b. Regarding digital identity implementation: How would requiring digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs) impact cybersecurity and data exchange?

Standardized identity verification would improve information security and bolster privacy. Today, there may be weak links in the identity verification process that allow malicious actors to gain access to PHI for either criminal or economic gain. For example, we often see third- or fourth-party impersonation of patients to request medical records. These parties then use this information for their own economic benefit or for illegitimate purposes—often without the patient even being aware that their PHI has been disclosed in their name.

3. Technical Standards and Certification

TD-4. How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

While the use of APIs is an important part of interoperability, the focus on the widespread adoption of publicly available APIs is, in our view, only part of the solution to interoperability. Specifically, CMS needs to ensure that the use of APIs promotes high-quality, usable data. Many vendors simply set up APIs to ensure they are minimally compliant with CMS and ASTP/ONC mandates; however, these vendors do not adequately ensure usability and data quality standards.

In addition, MRO encourages CMS to prioritize API development for clinical decision support and other patient treatment use cases. These APIs need to be well-functioning and widely adopted before expanding to other use cases. Fundamentally, patient treatment APIs are aligned with the needs of providers, who are essential actors in creating a high-quality healthcare data ecosystem. Use cases outside patient treatment can burden providers and may ultimately fail if they are not built upon a foundation of quality data.

TD-5. How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear a cost?

Providers and patients benefit from real-time access to medical information across different platforms, but we do not believe that a national Fast Healthcare Interoperability Resources® (FHIR) directory is the best way to enhance access. Instead, we believe that authentication and data quality are more critical to enhancing access to medical information than an endpoint directory. We are also concerned that the creation of a FHIR endpoint directory could inadvertently enable non-HIPAA-covered entities to phish for PHI for criminal or improper commercial purposes.

Regarding cost, non-HIPAA-covered entity stakeholders with an economic interest associated with the medical record should bear the cost of record fulfilment. We believe that patients, providers, and caregivers should continue to have access to medical records for treatment purposes for free or a very low cost. This is the framework largely in place today. If CMS moves forward with creating an endpoint directory, we ask that CMS make the directory available to all HIPAA Covered Entities at low or no cost.

TD-7d. Given improvements in language models, would you prefer a non-proprietary but less structured format that might improve data coverage even if it requires more processing by the receiver?

MRO does not believe that language models should be adopted to replace structured data formats. On the surface, because so much medical information is documented as text in a notes field, finding a standardized method to translate that text into useful data would have benefit. However, utilizing language models to extract data from text would raise additional privacy

concerns, as PHI would be used to train the models. By nature, language models are probabilistic, context-specific, and subject to change. We have serious questions and concerns around adopting language models, including:

- Who would adjudicate conflict when models are producing inconsistent and context-dependent results?
- Who would ensure that the model would not encode user PHI?
- Should the use of such models be limited to certain use cases, and who should enforce those use cases?
- If such a model is developed using PHI, is the model itself protected by HIPAA?

In addition, developing, licensing, and sufficiently testing large language models is an expensive endeavor and would introduce additional hurdles and costs for developers, which would slow down their pace of innovation.

TD-9a. Regarding certification of health IT: What are the benefits of redefining certification to prioritize API-enabled capabilities over software functionality?

MRO does not believe that there needs to be a binary choice between API-enabled capability and software functionality. Both API-enabled capability and software functionality affect data usability. For example, while certified health IT can support the exchange of non-health data (including supplemental benefit data) via APIs, if there is no mechanism within health IT certification to also ensure that users can discretely document such data within the certified products, those APIs would not actually be functional. More usable and accessible data would catalyze investment both in API development and in more capable software. Any specific use case may rely more on one than the other.

TD-9c. How could ASTP/ONC revise health IT certification criteria to require APIs to consistently support exchanging data from all aspects of the patient's chart (for example, faxed records, free text, discrete data)?

MRO recommends that ASTP/ONC modify health IT certification criteria to ensure effective and scalable document-based exchange. Today, much of the exchange of Consolidated Clinical Document Architecture (C-CDA) documents relies on older frameworks and technologies, such as SOAP web services and IHE profiles, rather than FHIR. We would support new criteria to ensure that certified health IT can exchange multiple types of C-CDA documents through FHIR APIs. This would allow innovators to tap into the existing FHIR endpoints that EHRs already offer for United States Core Data for Interoperability (USCDI) data and to streamline the retrieval of Continuity of Care Documents (CCDs), clinical notes, and other documents that are essential for individual access and treatment use cases.

More broadly, we encourage ASTP/ONC to promote the maturation of modern interoperability standards, such as FHIR and Clinical Decision Support (CDS) Hooks, as a fundamental step toward enhancing health IT certification. New and improved certification criteria should align with mature, vetted standards that ensure that health IT products offer consistent capabilities to innovators. An example would be API webhooks: we support introducing a new certification criterion to require certified health IT to offer event webhooks via a RESTful architecture. This requirement would provide better scalability and precision in interoperability by notifying third-party apps of important events for a patient, thus reducing the amount of repetitive polling that app developers must bake into their products and allowing the industry to accelerate its transition away from cumbersome Health Level Seven (HL7) version 2 interfaces. However, certified health IT products should not have free rein to implement webhooks in a proprietary format. Instead, ASTP/ONC should champion the maturation of FHIR Subscriptions and should establish them as the basis for required webhook capabilities.

TD-9d. What policy changes could CMS make so providers are motivated to respond to API-based data requests with best possible coverage and quality of data?

We believe that strong technology and business cases are essential to motivating providers to respond with the greatest coverage and quality. From a technology perspective, providers have embraced certified health IT and rely upon those products having robust capabilities. Certified health IT must provide API tools and functionality that providers can easily use to deploy, secure, monitor, and scale API coverage.

Interoperability must also be tied to a strong business case for providers. Providers must be incentivized to utilize their certified health IT's API tools to share data effectively, and these tools must preserve the pillars of data security and patient privacy. We believe, however, that regulating data quality and responsiveness at the provider level is unlikely to be successful, as this approach would be nearly impossible to police effectively. In our experience, providers are more motivated to share information using API-based technology when doing so is more cost effective. If APIs solve a problem for providers, and leveraging that API requires data to be entered more consistently and accurately, then providers will use that technology.

As ASTP/ONC encourages greater use of APIs, we note the inherent tension between greater use of automated data-sharing and data privacy protection. For example, it is not always the case that sensitive information (*e.g.,* substance use disorder data protected by 42 CFR Part 2, reproductive health information, or other PHI with elevated protections) is found in the fields where it is expected to be. Highly sensitive data could inadvertently be transmitted via an API, and it would be unclear who would bear responsibility for such a breach.

MRO does not believe that current standards of technology provide certainty that only the minimum necessary information is disclosed appropriately. Accordingly, CMS must balance the serious risks of inadvertent disclosure with the benefits of enhanced interoperability. Artificial intelligence may be able assist in the prevention of unintended disclosures in the future, but this capability is not yet mature to our knowledge.

TD-10. For EHR and other developers subject to the ONC Health IT Certification Program, what further steps should ASTP/ONC consider to implement the 21st Century Cures Act's API condition of certification that requires a developer's APIs to allow health information to be accessed, exchanged, and used without special effort, including providing access to all data elements of a patient's electronic health record to the extent permissible under applicable privacy laws?

While we appreciate the desire to promote the use of APIs, we believe the main issue is the enforcement of existing requirements. For example, some EHRs provide different levels of data access for a provider's Business Associate than for the provider customer, which creates service complications. Even when APIs are available, some EHR developers might use financial barriers, like excessive fees, to limit data access. Thus, we believe that CMS and ASTP/ONC should focus on enforcing the existing requirements and ensuring that economic interests do not limit data access.

4. Data Exchange

TD-15a, b. Regarding bulk FHIR APIs: How would increased use of bulk FHIR improve use cases and data flow? What are the potential disadvantages of their use?

MRO believes that increased bulk FHIR capabilities would open the door to key use cases like population health, data analytics, registries, and quality reporting. However, key technical limitations would hinder the effectiveness of bulk FHIR. First, bulk FHIR APIs today must be associated with a static group of patients that is defined in advance. There is little opportunity for managing a dynamic list of patients and pulling data for it over time. Developers must contact the EHR or provider to establish or update a group. Bulk APIs today generally have no programmatic way to do this without special effort.

Next, EHRs are often limited in the amount of data that their bulk FHIR exports can support, making scalability very poor and the usefulness of bulk FHIR relatively weak. If the amount of data that is allowed to be transmitted remains low, then the purpose of "bulk" data transfers will not be realized. Lastly, because greater interoperability and easier data exchange demand higher attention to security and precision, bulk FHIR APIs must be developed with robust options for filtering and precisely capturing only the minimum necessary information.

We also acknowledge that bulk FHIR could be vulnerable to potential abuse. There must be a means by which providers can verify that developers have appropriate access to the correct patient

population and data points and a means by which providers can monitor bulk FHIR use. We feel that our above suggestions—particularly a requirement for robust filtering options to enable precise capture of the minimum necessary information—can inform essential guardrails.

TD-16a, b. What are the tradeoffs of maintaining point-to-point models vs. shared network infrastructure? Do current rules encourage scalable network participation? What changes would improve alignment (for example, API unification, reciprocal access)?

MRO believes that many interoperability challenges can be effectively addressed through a robust point-to-point model. Point-to-point is not reliant upon shared rules and infrastructure. While rules may provide parties that are unfamiliar with each other a minimum set of parameters on which to exchange information, these rules can also limit the scope of data exchange between trusted exchange partners. Point-to-point models are easier to modify and tailor to situations in which data must be exchanged regularly between two trusted parties. Point-to-point models, done correctly, can be scalable and rapidly deployed.

Shared networks present challenges around data privacy and security, even if a network model is designed to create trust between exchange partners who may not be familiar with each other. In practice, we have found that privacy and security standards in shared networks are not robust enough, so data exchange is intentionally limited. A point-to-point model between two parties who are familiar with each other and who trust each other, however, has fewer limitations and has a greater level of quality and precision in the data exchanged. This greater data quality and precisions boosts satisfaction for the data requester, reduces the need to seek alternative paths to acquire the desired data, and enables more use cases with a richer data set.

TD-17. Given operational costs, what role should CMS or ASTP/ONC or both have in ensuring viability of healthcare data sharing networks, including enough supply and demand, that results in usage and outcomes?

MRO believes that, for networks to be useful, they must ensure privacy, security, and reliability. As echoed in our other responses, if network users are concerned that information sharing may result in a data breach, or in disclosure of sensitive patient information to non-HIPAA covered entities, they will not participate. Additionally, if users encounter barriers to getting useful information from their requests, they will not continue to use the network.

Solidifying the use cases around clinical decision support and other patient treatment activities is the best starting point for a viable healthcare data sharing network. To do so, data must be shared within the privacy and security framework of HIPAA. A data exchange designed around patient treatment would have more inherent trust than a data exchange designed for multiple use cases that may oppose each other. Additionally, we think the Agency should focus on ensuring

outcomes, not on developing the networks themselves. Ultimately, these networks need to stand on their own and competition may be necessary to solve the issues that plague data sharing networks today. The federal government can help create the right incentives and framework in which independent networks can thrive.

5. Compliance

TD-18a, b, c. Information blocking: Could you, as a technology vendor, provide examples for the types of practices you have experienced that may constitute information blocking. Please include both situations of non-responsiveness as well as situations that may cause a failure or unusable response? What additional policies could ASTP/ONC and CMS implement to further discourage healthcare providers from engaging in information blocking practices? Are there specific categories of healthcare actors covered under the definition of information blocking in section 3022(a)(1) of the Public Health Service Act (PHSA) that lack information blocking disincentives?

The concept of information blocking in the healthcare marketplace is complex due to the interplay between legitimate concerns about patient privacy and the overall market demand for information and interoperability. Additionally, EHR systems have highly sophisticated and complex gatekeeping mechanisms—such as permissions, access hierarchies, and embedded workflows—that determine what providers and their authorized Business Associates can access or do. For example, we have seen situations in which valid, authorized Business Associates are provided with lower-level data access permissions than their Covered Entity counterparts, resulting in unnecessary workflow constraints and other inefficiencies. These dynamics can stifle information flow, competition, and innovation.

Also, when providers act to ethically restrict access to sensitive PHI (particularly in scenarios involving vulnerable patients or contentious third-party requesters), they are increasingly met with threats of legal retaliation, often under the guise of alleged "information blocking." While the federal rules were designed to empower patients and to reduce opacity in health data exchange, such rules are frequently misinterpreted. Unfortunately, providers, already functioning within overburdened systems, are rarely equipped—legally, financially, or institutionally—to defend themselves against such threats.

We also frequently hear from our provider partners that the information blocking regulations are becoming increasingly complex and difficult to understand. We therefore encourage CMS and ASTP/ONC to consider ways to simplify the regulations when possible and to provide detailed guidance to assist providers in understanding and complying with the regulations. We recommend that CMS and ASTP/ONC revisit the existing information blocking rules with a focus on how to better enable the exchange of health data while maintaining rigorous protections for PHI, and in a way that drives fair competition and private sector innovation. Ultimately, CMS and ASTP/ONC

should endeavor to generate streamlined and clear information blocking regulations that are informed by the modern technology environment.

Conclusion

We appreciate the opportunity to provide our responses to the Health Technology Ecosystem RFI. If you have any questions about our responses, or need any additional information, please do not hesitate to contact me or Adrienne Morrell, Vice President, Government Affairs amorrell@mrocorp.com. Efforts to enhance interoperability across the healthcare ecosystem must emphasize data reliability, quality, privacy, and security. We look forward to continuing to partner with CMS and ASTP/ONC on these important efforts to improve data sharing, to accelerate progress toward a patient-centered health system, and to ensure the privacy and security of health information.

Sincerely,

Anthony Murray, CISSP
Chief Interoperability Officer
MRO Corporation